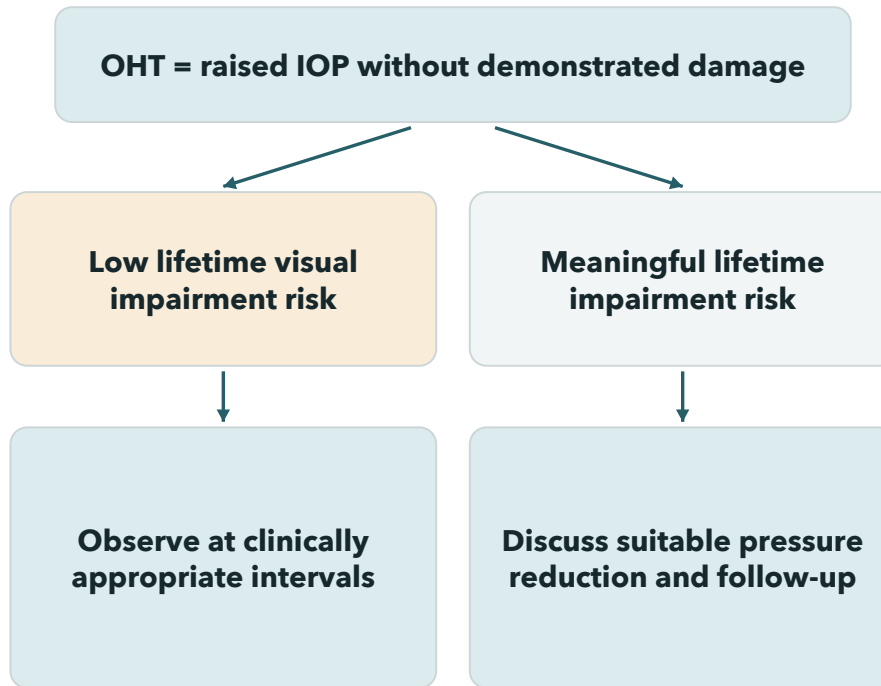


Glaucoma risk changes the pathway

Ocular hypertension and advanced optic nerve damage need different decisions.

Pressure alone does not decide treatment

A | From measurements to lifetime risk



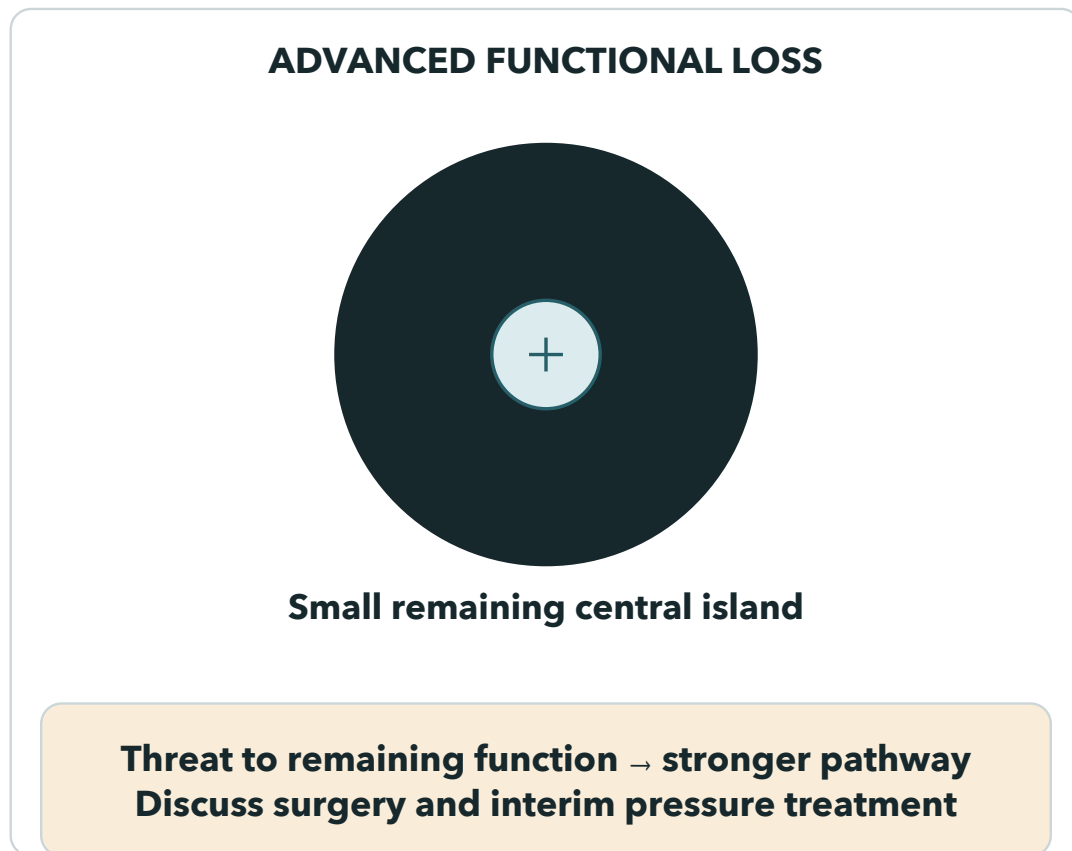
Original risk map. Evaluate pressure alongside the cornea, nerve, fields, family history and the patient's expected period at risk.

Known ocular hypertension

Assessment	Management implication
No nerve or field damage	OHT describes the state; it does not automatically mandate drops.
Low lifetime impairment risk	Appropriate observation and repeat eye assessment may be reasonable.
Meaningful lifetime risk	Discuss pressure reduction and suitable options using the local pathway.
New structural or field loss	Reclassify and reassess management; normal chart acuity is insufficient reassurance.

Advanced loss changes urgency and intensity

B | Protect the small remaining field



Original field-of-vision schematic. A central island represents advanced functional loss; it does not specify the patient's exact perimetry.

Do not apply the early-disease pathway automatically

NICE NG81 recommends offering glaucoma surgery with appropriate augmentation for suitable advanced chronic open-angle glaucoma, with interim pressure treatment while surgery is arranged. Discuss benefits, complications and the patient's preferences. If surgery is declined or unsuitable, choose an alternative plan with the specialist. The purpose is to preserve remaining function; treatment cannot promise recovery of established loss. Follow-up intensity should reflect control, progression and the risk to fixation, rather than central acuity alone. A low-risk older person with ocular hypertension may reasonably choose observation. Someone with a small central field island and a pressure above target needs a much more protective plan. Use the diagnosis, severity and lifetime risk together rather than treating a number in isolation.